Nivolumab · every 4 weeks · previously treated non-small-cell lung cancer (NSCLC)

Previously treated NSCLC Previously treated locally advanced or metastatic non-small-cell lung cancer (NSCLC). This patient suite is lung-focused; other shared protocol indications need their own counselling.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Nivolumab every 4 weeks

Active medicines: nivolumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Minimal emetic risk; no routine antiemetic prophylaxis or premedication. Prescribe rescue/support individually.
- Immune alert card and immediate symptom reporting.
- Blood count, kidney/liver tests and glucose before every 4-week cycle; thyroid once a month; cortisol if clinically indicated. Review cancer progression before cycle 4. Continue immune-toxicity monitoring for at least 5 months after the last dose.
- Effective contraception during treatment and at least 5 months afterwards; specialist breastfeeding discussion.
- Current NCIS medical-oncology guidance does not create a routine cytotoxic prescription for minimal-risk nivolumab monotherapy.
- Hormone replacement alone does not always mean nivolumab must stop. Your team checks symptoms and recovery and reviews any medicines that suppress the immune system before deciding whether treatment can continue or restart.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____